

Question:

A baby is born after the 32nd gestational week by cesarean delivery. The mother suffered from gestational diabetes; however, she had no other pregnancy-related diseases and was otherwise healthy. The baby has a blood pressure of 100/58 mm Hg, heart rate of 104/min, and oxygen saturation of 88%. The child has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis. The cyanosis is responding well to initial administration of oxygen. The nasogastric tube was positioned without problems. Which of the following is the most likely diagnosis?

Answer Choices:

- A. Neonatal respiratory distress syndrome (NRDS)
- B. Tracheoesophageal fistula
- C. Pneumonia
- D. Congenital heart anomaly with right-to-left shunt

Reasoning + Answer:

<think>

- The baby is born pre term (after 32 weeks), with no other pregnancy-related diseases except gestational diabetes .
- The baby shows signs of respiratory distress including tachypnea , subcostal and intercostal retractions , nasal flaring , and cyanosis .
- Cyanosis is responding to oxygen administration .
- Nasogastric tube positioning was uneventful , ruling out severe airway obstruction .
- There are no strong indicators for a congenital heart anomaly with right-to-left shunt or tracheoesophageal fistula .
- Pneumonia could be a cause , but the presence of respiratory distress without other signs of infection and the quick response to oxygen administration makes this less likely .
- The most likely diagnosis given the signs of respiratory distress and the context of prematurity is Neonatal Respiratory Distress Syndrome (NRDS).

- The most common cause of respiratory distress in premature infants is NRDS .
- NRDS occurs due to insufficient surfactant production in the lungs of the premature infant , leading to alveolar collapse .
- The quick response to oxygen administration supports the diagnosis of NRDS , as this condition is often responsive to supplemental oxygen .
- Other causes like pneumonia can present similarly , but the absence of other signs of infection and the quick response to oxygen administration make NRDS more likely .

- The most likely diagnosis is Neonatal Respiratory Distress Syndrome (NRDS).
- The other options (tracheoesophageal fistula , pneumonia , and congenital heart anomaly with right-to-left shunt) are less likely given the clinical presentation and the context of prematurity .

- The baby's condition is likely NRDS .
- The quick response to oxygen supports this diagnosis .
- The other options are less likely given the clinical presentation and the context of prematurity .

- The most likely diagnosis is Neonatal Respiratory Distress Syndrome (NRDS).
- This is the most common cause of respiratory distress in premature infants .
- The quick response to oxygen administration supports this diagnosis .
- The other options are less likely given the clinical presentation and the context of prematurity .

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<answer>A. Neonatal respiratory distress syndrome (NRDS)</answer>